Well-woman annual exam with heavy periods

A full doctor-patient conversation and the finished note it should produce, laid out in the Sully Well-Woman Annual Exam template. Use it to test an AI scribe, train new scribes, or see what a complete note captures.

Patient	42-year-old woman (fictional)
Visit	Established patient, preventive visit plus problem, in person
Length	About 15 minutes, 58 turns
Matching template	Well-Woman Annual Exam · sully.ai/templates/well-woman-annual-exam

A Visit transcript

Doctor	Hi Priya, good to see you again. It's been about a year.
Patient	Hi! Yeah, right on schedule for once.
Doctor	*(laughs)* I'll take it. So this is your annual, but before we go through the checklist, anything on your mind you want to make sure we get to?
Patient	Actually, yes. My periods have gotten really heavy. Like, the last six months or so.
Doctor	OK, let's start there then. Tell me what heavy looks like for you.
Patient	So the first two days I'm changing a super tampon and a pad every hour, hour and a half. I've bled through onto the sheets twice. I've had clots, like, the size of a quarter.
Doctor	How many days does the period last now?
Patient	Seven, sometimes eight. It used to be five.
Doctor	And how far apart are they? Still regular?
Patient	Regular. Every 27, 28 days. I track it on my app.
Doctor	Any bleeding between periods, or after sex?
Patient	No.
Doctor	Pain?
Patient	Cramping is worse than it used to be. The first two days I take Advil, like 600, a few times a day.
Doctor	Do you feel tired, short of breath going up stairs, lightheaded?
Patient	I'm exhausted. But I have two kids and a job, so I just figured, you know.
Doctor	Fair, but it could also be your blood count. Any cravings for ice? Some people with low iron start chewing ice.

Patient	*(pause)* Oh my God. Yes. I've been chewing ice all summer. I thought that was just a weird habit.
Doctor	It's a real clue. We'll check. Are you sexually active, and what are you using for contraception?
Patient	Yes, with my husband. He had a vasectomy two years ago.
Doctor	OK. Any chance of pregnancy otherwise?
Patient	No.
Doctor	Any hot flashes, night sweats, changes in mood around your cycle?
Patient	Not hot flashes. Maybe more irritable the week before.
Doctor	OK. Let's go through the rest quickly. Any new medical problems this year? Surgeries, hospital visits?
Patient	No. Same as always. Just the migraines, which are actually better.
Doctor	Still with aura?
Patient	Yeah, I get the zigzag lines first. Maybe once every two months now.
Doctor	Good to know, that matters for some treatment choices. Medications still just sumatriptan as needed?
Patient	And a multivitamin.
Doctor	Family history, anything new? Breast, ovarian, colon cancer?
Patient	My mom was diagnosed with breast cancer in March. She's 68. They caught it early, she had a lumpectomy.
Doctor	I'm sorry to hear that. How is she doing?
Patient	Good. She's doing radiation now. She's tough.
Doctor	I'm glad. We'll talk about what that means for your screening. Smoking, vaping, alcohol?
Patient	No smoking. Wine, maybe three glasses a week.
Doctor	Exercise?
Patient	I walk the dog. That's about it these days.
Doctor	And how's your mood been overall? Feeling down or hopeless, or losing interest in things?
Patient	Honestly, OK. Stressed with my mom. But OK.
Doctor	Do you feel safe at home?
Patient	Yes.
Doctor	OK. Your last Pap was three years ago, with HPV testing, both normal. So you're due for that today. Last mammogram was last November, normal. Let me examine you, and then we'll put it all together.
Doctor	*(exam)* Breasts, no lumps, no skin changes, no nipple discharge. No lymph nodes under the arms. Now the pelvic exam. You'll feel the speculum. *(pause)* Cervix looks healthy. I'm taking the Pap and HPV sample now. And now I'm going to feel the uterus and ovaries. Uterus feels mildly enlarged, a little irregular on the right side. Ovaries feel normal. OK, all done.
Patient	Irregular?

Doctor	A little bumpy, which most often means a fibroid. Fibroids are very common at your age and are a classic cause of heavier periods. They're benign. I'd like an ultrasound to see the size and where they sit, because that changes what we'd recommend.
Patient	OK.
Doctor	So here's the plan. Labs today: a blood count, iron and ferritin, and thyroid, because thyroid can also change periods. A pelvic ultrasound. If your iron is low, I'll start iron, and if it's really low we can do an iron infusion, which works faster.
Patient	OK.
Doctor	For the bleeding itself, there are good options, and the ultrasound will help us choose. One option is tranexamic acid, a pill you take only on the heavy days. It can cut the bleeding a lot. Another is a hormonal IUD, which thins the lining and many people end up with very light periods or none. Because of your migraines with aura, I would avoid the combined estrogen pill. The IUD is fine for you.
Patient	I've thought about the IUD. My sister has one and loves it.
Doctor	That's a great option for you. Let's get the ultrasound first and decide together. Now, about your mom. With one first-degree relative diagnosed at 68, I'll calculate your lifetime risk, and I'd like to refer you to genetic counseling to see if testing makes sense. Either way, keep yearly mammograms. Your next one is due this month.
Patient	OK. I'll schedule it.
Doctor	Vaccines: you're due for a tetanus booster this year, and a flu shot. We can do both today.
Patient	Sure.
Doctor	Any questions?
Patient	No, that was a lot but it makes sense. I can't believe the ice thing.
Doctor	*(laughs)* It's a classic. I'll message you with the labs.

B Finished note

Patient: Priya Raman (fictional), 42 F, G2P2. Established patient. Preventive visit with a separately documented problem.

Interval history: No new diagnoses, surgeries or hospitalizations. Migraine with aura, frequency decreased to ~1 every 2 months.

Menstrual history: Cycles regular, 27 to 28 days. Over ~6 months: duration increased from 5 to 7-8 days; heavy flow first 2 days (super tampon plus pad every 1 to 1.5 h), nocturnal soaking ×2, clots ~2.5 cm. Increased dysmenorrhea, ibuprofen 600 mg several times daily on days 1 to 2. No intermenstrual or postcoital bleeding. Fatigue and pagophagia (ice craving) over the summer. Mild premenstrual irritability. No vasomotor symptoms.

Sexual and contraceptive history: Monogamous with husband, partner vasectomy 2 years ago. Pregnancy not suspected.

Medications: Sumatriptan PRN, multivitamin.

Family history: Mother, breast cancer diagnosed at 68 (this year), lumpectomy, undergoing radiation.

Social: Non-smoker. Alcohol ~3 drinks/week. Walks dog. Mood stable, stress related to mother's illness. Feels safe at home.

Screening history: Pap with HPV co-test 3 years ago, normal. Mammogram 12 months ago, normal.

Examination: Breasts symmetric, no masses, skin changes or nipple discharge; no axillary lymphadenopathy. External genitalia normal. Cervix normal in appearance; Pap/HPV obtained. Bimanual: uterus mildly enlarged with irregular right contour, non-tender. Adnexa normal.

Assessment:

1. Annual well-woman exam.
2. Heavy menstrual bleeding with enlarged irregular uterus, suspected leiomyoma.
3. Suspected iron deficiency (fatigue, pagophagia, heavy menses).
4. Migraine with aura: estrogen-containing contraception contraindicated.
5. Family history of breast cancer, first-degree relative.

Plan:

- Pap and HPV co-test collected.
- CBC, ferritin, iron panel, TSH.
- Pelvic ultrasound (transabdominal and transvaginal).
- If iron deficient: oral iron vs IV iron depending on severity.
- Heavy bleeding options discussed: tranexamic acid on heavy days; levonorgestrel IUD (patient interested). Avoid combined hormonal contraception given migraine with aura. Decision after ultrasound.
- Lifetime breast cancer risk calculation; referral to genetic counseling. Annual mammogram, due now; patient to schedule.
- Tdap and influenza vaccines administered today.
- Results via patient portal; follow-up after ultrasound.

How to use this sample. Run the transcript through your scribe and compare its draft with the finished note. Check the details that are easy to miss: doses, which side of the body, what the patient said no to, and every follow-up step. For 536 more transcripts, many with audio and reference notes, see github.com/aniquethaplawala/open-clinical-encounters.

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